

February 23, 2026

Office of the Secretary
Department of Health and Human Services
Attention: RIN 0955-AA13

Submitted via regulations.gov

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

On behalf of National Council for Mental Wellbeing (National Council), thank you for the opportunity to comment on the Department of Health and Human Services (HHS) Request for Information (RFI), Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care, at 90 *Federal Register* ("FR") 60108 (December 23, 2025). National Council is a membership organization representing over 3,200 mental health and substance use treatment organizations and the more than 10 million children, adults, and families they serve across our nation.

We are grateful for HHS' efforts in continuing to strengthen the effective and responsible adoption of technology by providers and increasing beneficiary access to effective digital capabilities. Thoughtful efforts to drive adoption of safe and high-quality digital health products can offer opportunity to increase access to mental health and substance use care, relieve administrative burden, and decrease workforce burden for mental health and substance use care professionals, particularly in workforce shortage areas.

Please see below for our comments on questions provided in the RFI in the order they appear.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Adequate reimbursement pathways and resources are needed: The option to leverage safe, high-quality digital health products can offer opportunities to support care between mental health and substance use providers and their patients, particularly in rural areas where transportation barriers and provider shortages can make getting the right care at the right time difficult. While steps have been taken under Medicare to facilitate digital therapeutic reimbursement, it is key adequate reimbursement methodologies and resources exist for successful digital health product implementation.

Currently, mental health and substance use providers face obstacles to leveraging approved digital health products for patients, inclusive of technological barriers (e.g. limited smartphone access), lack of provider knowledge of available tools, lack of available psychoeducation for patients on the use of these tools, and lack of adequate reimbursement and resources for implementation.

Because mental health and substance use providers were left out of the HITECH Act of 2009, that provided significant funding for the adoption of EHRs, many behavioral health providers' information technology infrastructure and interoperability have been lacking. At this time, AI solution pricing is high, and one contributing factor is a lack of standardization of EHRs. As a result, AI software companies must spend resources becoming compatible with dozens of different EHR ecosystems, thereby increasing the cost.

Sustainable financing in the public sector, one of the largest customer bases, must be available to allow for adoption of AI innovation. Private sector developers also need clearer pathways for financial growth, particularly to galvanize private funding in early stages of company growth. This can only be achieved if the largest customer bases have the means for adoption and capacity for innovation.

Consideration of provider capacity is key, and innovation must not add to administrative burden:

Another barrier to adopting AI innovation includes adding additional capacity to already burdened and thinly stretched mental health and substance use staff. When staff are already stretched for time and resources in highly stressful environments, it can make it challenging to take risks on new technological innovations that require investment with limited resources and time to adopt. It is essential that new AI solutions are provider driven and do not add to administrative burden.

Often, AI technology is built without appropriate clinician input. To ensure tool efficacy, it's key that tools are adaptable to clinicians' complex needs and are able to smoothly integrate into existing workflows. And it is key to recognize that while some tools may work to decrease administrative burden, when implemented, such solutions may inadvertently increase it.

We suggest prioritizing AI usage that will reduce provider administrative burden before adding AI for clinical mental health and substance use care while further research is conducted on the implications for risk in exacerbating symptoms and ensuring patient privacy protections.

Ensure innovation is provider driven and supplements, not supplants, human-provided care: It's also key that furthering the use of digital technologies, such as AI, supplements, not supplants access to in-person, human-provided care, particularly in considering implications and treatment effectiveness for both the older adult population and youth and children.

More research is needed to understand implications and risks in applying AI in mental health and substance use clinical care – particularly in light of considering cases of AI induced psychosis, exacerbated depression symptoms, suicidality, and missed opportunities for crisis deescalation.¹ Moreover, because clinicians often work in high stress environments where close attention to someone's wellbeing is critical, implementation of innovative tools need to be well validated and

¹ <https://psychiatryonline.org/doi/10.1176/appi.pn.2025.10.10.5>
<https://www.ucsf.edu/news/2026/01/431366/psychiatrists-hope-chat-logs-can-reveal-secrets-ai-psychosis>
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reliable. While tools develop, there is a risk of a false sense of trust and overreliance when implementing tools that still need to be closely monitored in order to ensure patient safety.

Patient privacy protections should be uncompromised: AI implementation must adhere to and ensure patient privacy protection in accordance with federal and state laws. In considering the responsibilities providers have when recommending a digital product to be used by a patient, providers must also be able to understand data privacy protections and related legal requirements with assurances that the technology complies as well as data use policies for digital health products as a part of protecting patient privacy and ensuring transparency of any data use and storage policies as well as risks and benefits for the patient.

As discussed later, further provider education and guidance on 42 CFR Part 2 implementation is also needed. The implications and intersection of AI use and 42 CFR Part 2 requirements will also be important in development and on the ground implementation.

Education and training for patients and providers is needed: Finally, in order to provide safe, effective, and high-quality care, it's key providers receive training on digital products that are accurate, safe, efficacious, and reliable to actively work toward ameliorating any inaccuracy issues in delivering care. Additionally, it is key providers furnish psychoeducation on the use of these tools for patients prior to implementation in order to optimize effectiveness.

Additionally, patients must be able to have access to the devices needed for product use as well as adequate broadband, particularly in rural areas, where such technologies would require it in order for the service to be properly and effectively implemented.

Current entity restrictions must also be considered: We urge the Centers for Medicare and Medicaid (CMS) to consider engagement with states regarding any implementation issues in which managed care organizations (MCOs) restrict the use of claims provided to their value-based contracted healthcare providers and applications utilizing AI due to uncertainty regarding privacy, security, and liability risks.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Prioritize tools for and reduce administrative burden: As mentioned above, digital health products, especially those that automate systems or aid decision-making, must be accurate, safe, efficacious, and reliable. Such products must comply with all legal requirements, particularly privacy requirements, provide information and data use policies transparently, and they must not perpetuate any inaccuracy issues in delivering care. To improve administrative workflows, as well as to improve implementation more broadly, digital products should not contribute to administrative burden. Such products should be able to integrate, to the extent legally allowable, with the providers' existing tools in order to not create

duplicative efforts. Particularly with respect to reporting requirements providers face, we have heard from our members that reporting across systems that do not integrate creates a significant amount of provider burden. Additionally, digital health products should recognize the variability across state requirements providers must meet as well as the variability across products that address EHR systems. Finally, adequate infrastructure for both the patient and provider must be in place in order to achieve successful implementation.

Efficient funding mechanisms needed: Appropriate coding and reimbursement structures must be established to enable providers to leverage AI tools for the delivery of services. This should include accounting for usage of AI tools that support efficiencies in operations. Additionally, reimbursement for provider training and patient education will both be needed to account for work that ensures safe and effective implementation.

AI must be appropriately used in behavioral health care: Finally, as mentioned above, guardrails, that do not curb innovation and complies with all executive orders,² on use of AI for “therapy” will be needed to ensure AI is being used to augment, rather than replace in-person/human-provided clinical care. Training and oversight will also be needed to ensure there is not an overreliance on AI tools in decision making that could lead to escalation of symptoms or missed considerations, especially in complex cases that AI might not yet be able to navigate or have an adequate evidence base for.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Currently, reimbursement remains unclear for AI activities under existing CPT coding and other payment structures, including what qualifies as billable, what documentation and oversight are required, and how to ensure organizations and clinicians are not financially penalized for investing in and adopting these AI technologies. HHS will need to provide clear guidance and guardrails to both providers as well as technology vendors.

Moreover, guidance from HHS on appropriate data storage practices for providers and vendors is needed with clear delineation of liability protections when implementing AI solutions. HHS may consider evaluating state-level action currently happening and provide federal level guardrails.

Additionally, HHS will need to protect the scope of practice for mental health and substance use clinicians. For example, clear definitions and guidance must be provided to delineate types of solutions AI technologies provide (e.g. what can be called “therapy”). HHS should also look to consider quality

² <https://www.whitehouse.gov/presidential-actions/2025/12/eliminating-state-law-obstruction-of-national-artificial-intelligence-policy/>

standard guidelines to ensure that developers understand how well they are achieving benchmarks related to data integrity, testing, design features, and evaluation.

Finally, HHS will need to ensure rigorous safeguards for AI solutions are in place, particularly if solutions engage in furnishing emotional support. For example, this includes but is not limited to: protections for patients in crisis, suicidal and homicidal risk assessments, screening mechanisms for patients with delusions, and assurances that technology solutions comply with patient privacy protections. HHS can also help lift the burden off of providers by regulating accreditation, certification, and offering pre-vetted solutions that meet federal requirements.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Admission, Discharge, and Transfer (ADT) feeds would be made much more understandable and consumable with the application of AI. ADT feeds do not currently aggregate multiple admissions discharges and transfers into packaged episodes of care as people get transferred between different units of a hospital making it very difficult and time-consuming for outside providers to understand what an actual final discharge is occurring as opposed to transfer to another unit so that is single hospital admission generates multiple ADT notifications. Revising the Office of the National Coordinator for Health IT (ONC) and CMS guidance on ADT feeds in a manner that encourages application of AI to simplify them into aggregated episodes of care would greatly help outside healthcare providers.

Health information exchange (HIE) outputs would also be made much more understandable and consumable with the application of AI. Often finding the desired specific clinical information in an HIE query requires opening and reading large numbers of individual clinical documents. CMS could provide guidance and encourage HIEs to use AI to provide inquiry responses that are both more specific and more detailed to the query.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Promising performance outcomes: Thus far, ambient scribe tools have exceeded performance impact for community behavioral health providers - looking forward, there should be a focus on ensuring and improving quality of these tools while maintaining low barrier-to-entry cost for providers to adopt. Moreover, AI tools that can streamline burdensome processes in meeting regulatory requirements, intake, and assessment that may hold additional promise. In particular, screening and assessment tools that can screen for multiple conditions efficiently and make evidence-based recommendations in developing a treatment plan could be promising. VR or real time training for staff that has an evidence base may also assist in training and preparing for high intensity situations with clients.

Outcomes that have fallen short: Currently, the ROI has often fallen short given the burden and cost that implementation of new technology solutions require. At times it has been unclear that benefits or a positive ROI has materialized from administering new technology use – this outcome can often stem from a cost and/or implementation burden that is higher than potential benefits. Providers have also found false expectations around ease of implementation or interoperability of new tools. There have also been cases when a new technology solution implemented performs duplicative functions already being done through other technology, such as EHRs.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

The primary administrative hurdles to adoption continue to be financing and cost burden for these new technologies. It is imperative that providers of all types – licensed and non-licensed – are involved from the beginning in the adoption of AI tools within health care organizations. Ultimately, providers will play an essential role in the success (and thus clinical outcome success) of adoption, and time and resources are needed to do so. Organization and clinical leaders who oversee care models, workflows, and responsibility for assuming financial risk required to adopt and sustain new technology are also all critical stakeholders in employing AI solutions on the ground.

Private sector certification bodies, state level licensing bodies, as well as national bodies for nurses, doctors, psychologists, psychiatrists, social workers, therapists, and counselors will also all be essential to understanding and employing AI technologies across provider fields. This will also relate to information and education at colleges and universities while someone is on the path to becoming a provider.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Enhanced interoperability would allow for pooling of data across discrete payers to standardize reimbursement rates and advance value-based care arrangements, and allow patient care to extend outside the four walls of healthcare, in accordance with all laws and regulations, to community organizations and providers, social service systems, school systems, housing systems, criminal justice systems for example, where social and environmental factors are addressed, and lifestyle and functional medicine happen.

Providers could benefit from further education from CMS and ONC about the opportunities and requirements available under the relatively new CMS and ONC interoperability rules, in particular with regard to information blocking. Further guidance on how the interoperability rule interacts with HIPAA and 42 CFR part 2 is also needed. Currently, there is not adequate healthcare provider education and

training regarding the new requirements and financial penalties attached to information blocking. One of the most powerful opportunities of AI is its ability to aggregate and synthesize information from multiple sources, but that requires that those multiple sources do not engage in information blocking. Further guidance and support from CMS and ONC could help provider organizations adequately revise policies, procedures, forms, and staff training to integrate interoperability requirements with the updated requirements of 42 CFR part 2 as well as HIPPA requirements.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

AI may provide the opportunity to augment provider care and reduce administrative burden. However, it is critical to define and recognize that AI supported care is not the same as professional, human-delivered, formal behavioral health services. AI supported care should not attempt to and cannot replace the need for such human-delivered services. However, further efforts and additional research should prioritize leveraging both human-provided and AI-supported modalities of care such that we can create a behavioral health system that provides the right type and level of care, at the right time, in a way that can be constantly reevaluated and adapted for continuous improvement and effective and efficient implementation.

Further opportunities exist in AI adoption by use of tools that support accurate and tailored diagnosis and treatment plans, a reduction in the amount of times patients and caregivers need to answer provider questions, and improved accessibility across team-based care approaches that improves patient experience.

Patient and provider concerns also relate to privacy protections (e.g. what is done with the information and who else is seeing it). Privacy protections must comply with all applicable federal and state laws. Youth-specific privacy and safety protections must also be attended to for child and youth facing technology.

There are also concerns of algorithms driving clinical decisions that could increase the risk of provider overreliance or a false sense of security in the product, particularly where there could be reliability or safety issues with that algorithm.

Moreover, there are also concerns with patient-facing technologies that, without sufficient research and appropriate application, could exacerbate symptoms of depression, onset of psychosis, homicidality, suicidality, or escalate a crisis.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

HHS should prioritize further research on the impacts and implications of clinical decision supported AI use in mental health and substance use care. Currently, there does not appear to be a substantial, robust body of literature to understand the complexity of the implications that likely exist when applying AI to clinical mental health or substance use care. There must also be further evaluation of the risk of clinicians overreliance as AI solutions are adopted. Research must also better understand the interaction between implementing AI-supported care and human-delivered services. Further research topics that will support the field and potential for adoption of AI include research on: screening and assessment, effect of AI chatbot use for emotional support and light-touch interventions, existing use and efficacy of AI community mental health and substance use settings.

National Council appreciates the opportunity to provide these comments. We welcome any questions or further discussion about the recommendations described here. Please contact Reyna Taylor at ReynaT@thenationalcouncil.org. Thank you for your time and consideration.

Sincerely,

A handwritten signature in black ink that reads "Charles Ingoglia". The signature is written in a cursive, flowing style.

Charles Ingoglia, MSW
President & CEO
National Council for Mental Wellbeing